

**PRODUCT**

MaternaLink (in development) — 8-Quarter Roadmap (Q4 2026 – Q3 2028)

Vytalix company-build documentation · reference
MATERNALINK_ROADMAP

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

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Owner: CPO/CTO, Vytalix · **Status:** PROPOSED v0.1 · **Date:** 7 September 2026 · **Governing files:**

/FACTS_BASE.md ; /docs/04_MATERNALINK_PRODUCT_STRATEGY.md (\$22–\$23 summarise this roadmap);
/product/MATERNALINK_PRD.md ; /product/MATERNALINK_MVP_SPEC.md .

Every milestone here is a plan, not a commitment or a record. Dates assume incorporation, IP resolution and funding in Q4 2026 (FACTS_BASE A3/A7); slippage in any gate moves everything after it. Sources cited are the six regulatory searches used for this batch; everything else is ESTIMATE or TO VALIDATE.

Executive view (5 lines)

1. Eight quarters take MaternaLink (in development) from a concept and an unaudited prototype to a **v2.x product with evidence from one UK service evaluation, one UK feasibility study, one Nigeria pilot and a multi-site formal evaluation under way** — and a documented decision on whether any v2 monitoring feature is a medical device.
2. The roadmap is **gate-driven**: G0 IP/ownership → G1 qualification decision → G2 safety case → G3 DPIA → G4 DTAC 2.0 → G5 Cyber Essentials/Plus → G6 DSPT → G7 NDPC registration → G8 device decision (v2) → G9 UKCA Class I if applicable → G10 ISO 13485 decision for v3; no external claim is made ahead of the gate that supports it.
3. **Evidence stages define permitted language**: service evaluation ("in use at", usability, operational metrics) → feasibility ("feasible and acceptable") → pilot ("engagement/adherence in context") → formal evaluation ("associated with" process outcomes) — mortality and morbidity claims are never permitted without a powered, independent study, which is beyond this horizon.
4. **Hiring is triggered by evidence and cash, not by the calendar**: first engineers on funding of the MVP; a clinical lead on the first evaluation site; a Nigeria country lead on a funded pilot; a regulatory/QA lead only if G8 concludes "device".
5. Dependencies outside Vytalix's control dominate: an NHS evaluation site, a Nigeria partner, gateway/sender-ID approvals, HRA/REC and NHREC timelines, and grant cycles — each quarter therefore names a fallback.

1. Timeline at a glance

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title MaternaLink (in development) – 8 quarters (PROPOSED)
dateFormat YYYY-MM-DD
axisFormat %b %y
section Product
Discovery + build weeks 1-6 :p1, 2026-10-01, 90d
MVP complete (weeks 7-16), alpha :p2, 2027-01-01, 90d
v1.0 live (UK eval) + v1.1 WhatsApp/Yoruba/voice :p3, 2027-04-01, 91d
v1.2 USSD, DHIS2 export :p4, 2027-07-01, 92d
v2.0 FHIR read facade, PDS (TO VALIDATE), flagging decision :p5, 2027-10-01, 92d
v2.1 partner access, MH instruments (if cleared), Hausa/Igbo/Pidgin :p6, 2028-01-01, 91d
v2.2 EPR export interfaces, v3 research design :p7, 2028-04-01, 91d
v2.3 hardening; v3 retrospective modelling (under approvals) :p8, 2028-07-01, 92d
section Evidence
Discovery interviews (20+10 per market) :e1, 2026-10-01, 75d
UK service evaluation protocol :e2, 2027-01-15, 60d
UK service evaluation (n 50-150) :e3, 2027-04-15, 150d
Nigeria pilot design + ethics :e4, 2027-04-01, 120d
Nigeria pilot (1-2 LGAs, 500-2,000) :e5, 2027-08-01, 365d
UK feasibility study (2 sites) :e6, 2027-10-15, 180d
UK formal evaluation (multi-site) :e7, 2028-04-15, 180d
section Gates
G0 IP resolved; prototype audited :milestone, g0, 2026-11-30, 0d
G1-G3 qualification, safety case v1, DPIA :milestone, g1, 2027-03-15, 0d
G4-G5 DTAC 2.0 form, Cyber Essentials :milestone, g4, 2027-03-31, 0d
G6 DSPT submission; G7 NDPC (if req.) :milestone, g6, 2027-06-30, 0d
G5+ Cyber Essentials Plus :milestone, g5, 2027-09-30, 0d
G8 device qualification decision (v2) :milestone, g8, 2027-12-15, 0d
G9 UKCA Class I (if applicable) :milestone, g9, 2028-03-31, 0d
G10 ISO 13485 decision (v3) :milestone, g10, 2028-09-30, 0d

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2. Quarter-by-quarter plan

Legend: **Product** (what ships), **Evidence** (what is learned and what may then be said), **Regulatory/assurance gates**, **Commercial** (from strategy §23), **Dependencies**, **Hiring trigger**, **Fallback**.

Q4 2026 (Oct–Dec) — Foundations

Track	Milestones
Product	Discovery (20 women + 10 professionals per market); PRD v0.2 signed off; design system and IA; architecture ADRs; Terraform/CI/CD/environments; build weeks 1–6 (auth, tenancy, enrolment, consent, schedule, reminders, gateway adapters)
Evidence	Discovery synthesis: validated personas, SLA/service-hour defaults, channel preferences; permitted claims : none about the product; "we are building, informed by interviews with N women and N professionals"
Gates	G0 IP/ownership documented; prototype audit complete (reuse vs rebuild decided); CSO contracted; DCB0129 hazard log opened; DPIA v0.1; ICO registration on incorporation; clinical advisory group formed
Commercial	Advisory/consulting revenue funds runway (FACTS_BASE A6); Ekiti status call and mDoc complementarity call; two alternative states scored; one UK service-evaluation LOI targeted
Dependencies	Incorporation; David Agunede agreement; funding decision for the build (UK-led £220k–£320k vs blended £140k–£210k, ESTIMATE); access to interviewees via community organisations (no relationships exist — TO VALIDATE)

Track	Milestones
Hiring trigger	MVP funding confirmed → contract tech lead + 2 engineers + 0.5 designer; CSO 0.2 FTE; fractional DPO
Fallback	If funding is only partial: build the Nigeria SMS-cohort slice first (enrolment, schedule, SMS, defaulter list) because it needs the least UI and can earn an NGO programme licence sooner

Q1 2027 (Jan–Mar) — MVP complete

Track	Milestones
Product	Build weeks 7–16: messaging + SLA engine + rota; diary; content library (≈40 UK + ≈20 NG items; Yoruba subset if pilot confirmed); offline sync; console; handover; escalation; defaulters; dashboard; admin/audit. Internal alpha with synthetic data; UAT with the evaluation site's digital midwife
Evidence	Service-evaluation protocol agreed with the UK site (service evaluation ≠ research: confirm with the HRA decision tool — TO VALIDATE — so no REC approval is required, but trust governance and Caldicott approval are); baseline operational metrics defined (contact completion, acknowledgement times). Permitted claims: "MVP complete; entering a service evaluation"
Gates	G1 qualification decisions documented per Must feature (not a device — TO VALIDATE with counsel); G2 clinical safety case report v1 signed by CSO; G3 DPIA v1 approved; DPA with the site; G4 DTAC 2.0 form completed (the form in force from 6 April 2026 — sources: https://www.burges-salmon.com/articles/102mnjh/new-nhs-digital-technology-assessment-criteria-what-health-tech-suppliers-need-t/ and https://digitalregulations.innovation.nhs.uk/regulations-and-guidance-for-developers/all-developers-guidance/using-the-digital-technology-assessment-criteria-dtac/); G5 Cyber Essentials (basic) certified; penetration test passed; WCAG 2.2 AA audit passed
Commercial	Grant applications (Innovate UK, SBRI Healthcare where a call fits — TO VALIDATE); one Nigeria NGO programme LOI; gateway contract and sender-ID registration
Dependencies	Evaluation-site sign-off on DTAC and DCB0160 (the trust's side); gateway sandbox; content licensing answers
Hiring trigger	Evaluation site confirmed → contract a clinical lead/digital midwife advisor (0.2–0.4 FTE); support engineer 0.3 FTE from go-live
Fallback	If no UK site by end of quarter: go live first with a private maternity/independent midwife group (strategy \$9) or the NGO cohort

Q2 2027 (Apr–Jun) — v1.0 live; v1.1

Track	Milestones
Product	v1.0 live in the UK service evaluation (one community team); v1.1: WhatsApp Business Platform channel (via BSP), Yoruba content library and UI strings, voice reminders (pre-recorded), pre-visit summary, bulk CSV import hardening
Evidence	UK service evaluation runs (n ≈ 50–150 women; ESTIMATE); measures: usability (SUS), acknowledgement times, contact completion, handover usage, safety events, staff time; Nigeria pilot designed with the partner (theory of change, indicators aligned to DHIS2, ethics application to NHREC/state committee). Permitted claims after this quarter: "in use in a service evaluation at [site, with permission]"; descriptive usage numbers; no effectiveness language
Gates	G6 DSPT submitted (category and cycle TO VALIDATE — 2025-26 v8 deadline was 30 June 2026, so this is the 2026-27 cycle; source: https://www.dsptoolkit.nhs.uk/News/161); G7 NDPC registration as a data controller/processor of major importance if thresholds apply (health sector; >200 data subjects in 6 months reported — TO VALIDATE: https://ndpc.gov.ng/wp-content/uploads/2025/07/NDP-ACT-GAID-2025-MARCH-20TH.pdf); WhatsApp DPIA addendum; sender ID live
Commercial	First paid pilot (private maternity or NGO programme licence, £10k–£40k ESTIMATE); employer-benefits conversations
Dependencies	Site go-live readiness (DCB0160); NDPC process time; BSP onboarding

Track	Milestones
Hiring trigger	Paid pilot + Nigeria partner signed → Nigeria country lead (contract), community engagement/content coordinator (Yoruba)
Fallback	If NHREC/state ethics is slow: start the Nigeria cohort as a service delivery pilot with routine monitoring (no research claims) while the evaluation protocol is approved

Q3 2027 (Jul–Sep) — v1.2; Nigeria pilot starts

Track	Milestones
Product	v1.2: USSD menus; DHIS2 aggregate export; referral notification to receiving facility; CHEW field mode for defaulter tracing; partner-access design (not release); performance and cost tuning for SMS volume
Evidence	Nigeria pilot starts (1–2 LGAs; 500–2,000 women; ESTIMATE) with the implementing partner; UK service evaluation interim report; UK feasibility-study protocol (2 sites) drafted for HRA/REC. Permitted claims: service-evaluation results as descriptive findings (e.g., "median acknowledgement time X hours"); "pilot under way in [state] with [partner]" only with their written permission and never "government partner" (FACTS_BASE §4)
Gates	G5+ Cyber Essentials Plus (ESTIMATE £1,500–£3,000 + VAT: https://www.figggroup.co.uk/blog/cyber-essentials-vs-cyber-essentials-plus-cost); Nigeria ethics approval for the pilot evaluation; safety case v1.1 (new channels); annual pen-test scheduling
Commercial	Nigeria pilot funded (donor or state; £0.1m–£0.3m ESTIMATE); G-Cloud listing preparation (TO VALIDATE route); second UK site conversations
Dependencies	Pilot funding; USSD short-code lease; state data-sharing agreement
Hiring trigger	Pilot funded → 2 Nigeria-based field/support staff via the partner (not Vytalix payroll); +1 engineer if two markets are live
Fallback	If pilot funding slips: run a smaller NGO cohort (≤500) on the programme licence

Q4 2027 (Oct–Dec) — v2.0; device decision

Track	Milestones
Product	v2.0: FHIR UK Core read facade (Patient, Encounter, Appointment resources) for future EPR integration; PDS demographics lookup (NHS API onboarding TO VALIDATE); rule-based flagging of self-recorded observations designed but not released pending G8; audit/report improvements from evaluation feedback
Evidence	UK feasibility study begins (2 sites; mixed methods; pre-registered; HRA/REC approval); Nigeria pilot monthly monitoring. Permitted claims: unchanged; feasibility results not yet available
Gates	G8 documented qualification decision on rule-based flagging and scored questionnaires with regulatory counsel: if "device", Class I route (UKCA self-declaration, MHRA registration, technical file, PMS plan under the post-market surveillance regime in force since June 2025 with 15-day serious-incident reporting — sources: https://www.qualio.com/blog/mhra-s-2026-devices-reform-what-changes-and-when and https://patientguard.com/the-mhra-2026-regulatory-roadmap-explained/); if "not device", release under the safety case with explicit intended-use wording
Commercial	First NHS trust paid-contract discussion post-evaluation; G-Cloud listing (TO VALIDATE); seed-round preparation on evidence
Dependencies	HRA/REC timeline (ESTIMATE 3–6 months); NHS API onboarding; counsel availability
Hiring trigger	G8 = device → contract a regulatory/QA lead (0.3 FTE) and adopt IEC 62304/ISO 14971 processes for that module; evidence lead (0.5 FTE) for the feasibility study
Fallback	If G8 is unresolved: v2.0 ships without flagging; the roadmap item moves one quarter, not the release

Q1 2028 (Jan–Mar) — v2.1

Track	Milestones
Product	v2.1: partner/companion access with consent and safeguarding controls; mental-health instruments (EPDS/Whooley/GAD-2) only if G8 cleared them ; Hausa, Igbo and Pidgin libraries (SMS/voice-first); transport/emergency-fund tracking (Nigeria)
Evidence	Nigeria pilot interim results (engagement, contact completion vs baseline, SMS reach); UK formal-evaluation protocol (multi-site; stepped-wedge or cluster design TO VALIDATE with a methodologist). Permitted claims: pilot interim descriptive results with the partner's permission; "feasibility study under way"
Gates	G9 UKCA Class I registration if applicable; MHRA annual PMS fees apply from April 2026 for registered devices (£300 per applicable GMDN category for 2026–27 — source: https://patientguard.com/the-mhra-2026-regulatory-roadmap-explained/ — TO VALIDATE); safety case v2; DPIA v2 (partner access); DSPT re-submission cycle
Commercial	2–3 UK sites; one state-scale agreement in Nigeria targeted (10k+ women); seed round on evidence
Dependencies	Translation partners for Hausa/Igbo; northern-state partner for Hausa content; investor appetite
Hiring trigger	Seed round closed → first employed engineers (2–3), product manager, customer success lead (UK), Nigeria country manager (employed)
Fallback	If the seed round slips: hold v2.1 to partner access + one language; extend services revenue

Q2 2028 (Apr–Jun) — v2.2; formal evaluation starts

Track	Milestones
Product	v2.2: EPR export interfaces (vendor-supported; availability TO VALIDATE per vendor); handover FHIR export; v3 research design (data specification, protocol, fairness analysis plan); analytics warehouse hardening
Evidence	UK formal evaluation starts (multi-site; process outcomes: contact completion, acknowledgement times, handover completeness, women's reported experience; not powered for mortality); Nigeria scale-up 5k–10k women with the partner. Permitted claims: "formal evaluation under way"; feasibility results as published
Gates	Data-sharing agreements for v3 research; HRA/REC application for v3 data access; NHREC application (Nigeria teaching hospital)
Commercial	State-scale contract signed (ESTIMATE); NHS contracts 2–3; ARR scenario building toward £0.4m–£0.9m by Q3 2028 (scenario, not forecast; strategy \$23)
Dependencies	Academic partner for the evaluation (no relationship exists — TO VALIDATE); EPR vendor cooperation
Hiring trigger	Formal evaluation funded → research/data scientist (v3, 1 FTE) reporting to the evidence lead; clinical safety team to 0.4 FTE
Fallback	If no academic partner: run a rigorous internal evaluation with an independent evaluator (e.g., a university consultancy) and label it accordingly

Q3 2028 (Jul–Sep) — v2.3; v3 research begins

Track	Milestones
Product	v2.3: hardening, cost optimisation, multi-region readiness (Nigeria cell live if contracted), accessibility re-audit; v3 retrospective modelling under approvals in a separated research environment (no production patient data without the approved data flow)
Evidence	Interim evaluation reports (UK and Nigeria); v3 pre-registered protocol; TRIPOD+AI reporting plan. Permitted claims: still no prediction or outcome claims; research described as research

Track	Milestones
Gates	G10 ISO 13485 QMS decision for v3 (if the research suggests a viable device, begin QMS; ESTIMATE 24–36 months and £1.5m–£4m to a clinical claim, strategy S8); annual pen test; CE Plus renewal; DSPT
Commercial	Series-A preparation on formal-evaluation interim data; expansion market scored (second African market)
Dependencies	Research approvals; investor appetite for deep-tech optionality
Hiring trigger	G10 = proceed → regulatory affairs lead (1 FTE), quality manager (0.5–1 FTE)
Fallback	If v3 evidence is weak: keep v3 as an academic collaboration and concentrate on v2 scale — this is an acceptable outcome, not a failure

3. Evidence-generation plan and permitted claims

flowchart LR

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A[Discovery<br/>Q4 2026] --> B[Service evaluation<br/>UK, Q2-Q3 2027]
B --> C[Feasibility study<br/>UK 2 sites, Q4 2027-Q1 2028]
A --> D[Nigeria pilot<br/>Q3 2027-Q3 2028]
C --> E[Formal evaluation<br/>multi-site, Q2 2028+]
D --> E
E --> F[v3 research track<br/>retrospective modelling under approvals]

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Stage	Design	Governance	Questions answered	Claims permitted afterwards (with data, dates, and site permission)	Claims never permitted at this stage
Discovery	Interviews, observation, co-design	Consent for interviews; no patient data	Needs, channels, language, SLA norms	"Designed with input from N women and N professionals"	Any product performance
Service evaluation	Single site, before/after operational metrics, usability	Trust service-evaluation governance (HRA decision tool → not research, TO VALIDATE); DCB0160 at the trust; DPIA	Is it usable, safe in operation, adopted by staff? What are acknowledgement times and contact completion?	"In use in a service evaluation at [site]"; usability scores; descriptive operational metrics	Effectiveness; "improves outcomes"; "validated"
Feasibility study	Two sites, mixed methods, pre-registered	HRA/REC approval; sponsor; PPI involvement	Is a formal evaluation feasible (recruitment, retention, data completeness, acceptability)?	"Feasible and acceptable in a study of N women across two sites"	Efficacy; "proven"
Nigeria pilot	1–2 LGAs, cohort with comparison LGAs where possible; DHIS2-aligned indicators	NHREC/state ethics; state data-sharing agreement; NDPA compliance	Reach, engagement, contact completion vs baseline, defaulter tracing time, cost per woman	"Pilot in [LGAs] with [partner]: reach X%, contact completion +Y points vs baseline" (as observed, with caveats)	"Reduces maternal mortality"; "government partner"

Stage	Design	Governance	Questions answered	Claims permitted afterwards (with data, dates, and site permission)	Claims never permitted at this stage
Formal evaluation	Multi-site, stepped-wedge or cluster-randomised (design TO VALIDATE), pre-registered, independent analysis	HRA/REC; sponsor; trial registration; data monitoring	Effect on process outcomes and experience measures; safety events; cost	"Associated with [effect] on [process outcome] in a multi-site evaluation"	Mortality/morbidity reduction unless the study is powered for it (out of horizon)
v3 research	Retrospective model development under data-sharing agreements; fairness analysis	HRA/REC; NHREC; DPIA; no production data without approval	Is a multimodal signal detectable and fair?	Research findings in publications only	Any clinical claim; any product feature

NICE Evidence Standards Framework mapping (TO VALIDATE): v1 → Tier B (communication and simple monitoring-support); v2 flagging and v3 → Tier C with higher evidence requirements.

4. Regulatory and assurance gates

Gate	What must be true	Evidence artefact	Owner	Quarter
G0 IP and prototype	Written IP assignment/licence covering code, brand and proposals; prototype audit complete; naming clearance (VITALIX/VYTALYX conflicts in FACTS_BASE)	Signed agreement; audit report	Founder	Q4 2026
G1 Qualification	Intended-use statement adopted; per-feature qualification decisions recorded (MHRA guidance on software apps and on crafting an intended purpose — strategy §21.1)	Qualification decision log; counsel letter	CTO + counsel	Q1 2027
G2 Clinical safety	DCB0129 hazard log complete; Clinical Safety Case Report v1 signed by CSO; deploying site's DCB0160 done	CSCR v1	CSO	Q1 2027
G3 Data protection	DPIA approved; DPAs; RoPA; ICO registration; privacy notices in the woman's languages	DPIA v1	DPO	Q1 2027
G4 DTAC 2.0	Form completed across the five areas; evaluating trust assesses before the pilot (DTAC applies before pilots as well as procurement)	DTAC form + evidence	CTO	Q1 2027
G5 / G5+ Cyber Essentials / Plus	Five controls evidenced; Plus tested by an assessor	Certificates	CTO	Q1 2027 / Q3 2027
G6 DSPT	Supplier submission for the current cycle (v8 CAF-aligned; Category 3 evidence set)	DSPT status	CTO + DPO	Q2 2027 (then annual)
G7 NDPC	Registration as a data controller/processor of major importance if thresholds apply; DPO named; NDPA DPIA	NDPC certificate	Nigeria lead + counsel	Q2 2027
G8 Device decision (v2)	Documented decision on flagging/scoring with counsel; if device, Class I route resourced	Decision memo	CTO + CSO + counsel	Q4 2027

Gate	What must be true	Evidence artefact	Owner	Quarter
G9 UKCA Class I	Technical file; UKCA self-declaration; MHRA registration; PMS plan (Part 4A regime; 15-day serious-incident reporting; annual PMS fees TO VALIDATE)	Registration	Regulatory lead	Q1 2028 (if applicable)
G10 ISO 13485 (v3)	Decision to build a QMS for a Class IIa+ prediction device; budget approved	Board decision	CEO + CTO	Q3 2028

Note on CE/UKCA: EU-MDR CE-marked devices are accepted in Great Britain until 30 June 2030 under current transitional rules (source: <https://trustedtracemed.com/resources/ukca-medical-software-2026.html> — TO VALIDATE); this matters only if MaternaLink ever pursues EU marking first.

5. Dependencies map

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flowchart TD
    IP[G0 IP resolved] --> BUILD[MVP build]
    FUND[MVP funding: services + grants + angels] --> BUILD
    BUILD --> ASSUR[G1-G5 assurance pack]
    SITE[UK evaluation site LOI] --> ASSUR
    ASSUR --> LIVE[v1.0 live UK]
    PARTNER[Nigeria partner + state/LGA agreement] --> PILOTDESIGN[Pilot design + ethics]
    GATEWAY[Gateway + sender ID + NDPC] --> PILOT[Nigeria pilot]
    PILOTDESIGN --> PILOT
    PILOTFUND[Pilot funding] --> PILOT
    LIVE --> SE[Service evaluation results]
    SE --> FEAS[Feasibility study]
    FEAS --> FORMAL[Formal evaluation]
    PILOT --> FORMAL
    SE --> SEED[Seed round]
    PILOT --> SEED
    SEED --> HIRE[Employed team]
    G8[G8 device decision] --> V2REL[v2 flagging release]
    FORMAL --> V3[v3 research]
  
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6. Hiring triggers (summary)

Trigger	Role(s)	Model	Cost ESTIMATE
MVP funding confirmed (Q4 2026)	Tech lead; 2 engineers; 0.5 designer; 0.5 QA; CSO 0.2 FTE; fractional DPO	Contract	See MVP spec §13.2
Evaluation site confirmed (Q1 2027)	Clinical lead / digital midwife advisor 0.2–0.4 FTE; support engineer 0.3 FTE	Contract	£3k–£6k/month
Nigeria pilot funded (Q3 2027)	Country lead (contract); field staff via partner; +1 engineer	Contract/partner	£4k–£8k/month
G8 = device (Q4 2027)	Regulatory/QA lead 0.3 FTE	Contract	£3k–£5k/month
Feasibility study funded (Q4 2027)	Evidence lead 0.5 FTE	Contract/academic secondment	£3k–£5k/month

Trigger	Role(s)	Model	Cost ESTIMATE
Seed round closed (Q1 2028)	2–3 employed engineers; product manager; customer success (UK); Nigeria country manager	Employed	£35k–£45k/month payroll (ESTIMATE)
Formal evaluation funded (Q2 2028)	Research/data scientist (v3)	Employed	£6k–£8k/month
G10 = proceed (Q3 2028)	Regulatory affairs lead; quality manager	Employed	£10k–£15k/month

7. What would change this roadmap

Signal	Change
Discovery shows women in the UK site overwhelmingly want WhatsApp over an app	Pull WhatsApp into MVP scope in place of web push; DPIA earlier
Ekiti ministry confirms a defined gap alongside mDoc	Nigeria pilot in Ekiti LGAs in Q2–Q3 2027 with Yoruba content prioritised
A northern state partner emerges first	Hausa content moves to v1.1; Yoruba to v1.2
Counsel judges the handover summary or diary display a device	Redesign those features (e.g., remove diary values from staff view) before go-live; add a Class I route to Q2 2027
A grant is won that requires an academic partner	Feasibility study moves earlier and becomes the evaluation vehicle
Prototype audit is clean and IP is assigned	Reuse of UI components; build shortened by ~2 weeks (ESTIMATE)

Priorities / Risks / Next actions

Priorities

1. Close G0 in Q4 2026; nothing on this roadmap is real without it.
2. Secure the UK evaluation site and the Nigeria partner in parallel with the build, not after it.
3. Treat the assurance pack (G1–G5) as MVP deliverables with the same status as code.
4. Design v2 monitoring features early enough that the G8 decision is made on documents, not on shipped code.
5. Keep v3 explicitly a research option in every external document.

Risks

Risk	Effect on roadmap	Mitigation
Funding shortfall for the MVP	Q1–Q2 2027 slip by 1–3 quarters	Blended team; Nigeria-slice-first fallback; grants
No evaluation site	v1.0 live slips; evidence chain delayed	Private maternity/NGO alternative; artefact-led selling
Ethics timelines (HRA/REC, NHREC)	Feasibility/pilot evaluation slip	Service-delivery pilot with routine monitoring first; early applications
G8 concludes "device" for core v2 items	+1–2 quarters and regulatory cost	Class I route budgeted; features designed as separable modules

Risk	Effect on roadmap	Mitigation
Partner or state political change in Nigeria	Pilot interruption	Two-state pipeline; NGO route independent of state
Over-claiming under investor pressure	Credibility and regulatory exposure	This document's permitted-claims table enforced in all materials

Next actions

- Founder: G0 actions; site and partner LOIs; funding decision.
- CPO/CTO: convert Q4 2026 into a sprint plan; appoint CSO; draft qualification-decision log template; start DTAC 2.0 form from the PRD.
- Clinical lead (to recruit): draft the service-evaluation protocol with the site; content licensing.
- Commercial: pilot funding pipeline (donors, state, NGO); grant calendar with deadlines; investor update template that uses only permitted claims.